

Week 13: Video Transcript

Hello everyone. Welcome to week 13. Congratulations on making it to phase two. I will be talking you through today's class, getting us ready, excited, and clear on how to begin working with your practice client, hosting and facilitating your first session. We'll also be discussing the bread and butter of what to expect moving into Jai's facilitation coaching portion of the program.

Let's get started. So let's discuss, and all of these slides will be following along with your weekly workbooks. So you can go ahead and have that workbook out with you if you'd like. Or you can put your full presence here to my voice and what it is that I am discussing. Feel free to take notes or not. In general, with every session for session here and evermore working with your clients, it is suggested and encouraged that you practice, experiment, and explore: How do I best prepare myself for a session? How do I best stay in myself during a session? And how do I best care for myself after a session?

There are suggestions, and we can share amongst each other, amongst our classes. I can share with you what I do, but ultimately it's going to be a personal practice of trying one thing, trying another, and finding your groove after a few rounds and a few practices, and taking the utmost care of yourself in this profession. This can require, as if you are going to work out for an hour, and you're working out the muscles of presence, attunement, self-regulation of your own emotions, staying in your wise prefrontal cortex rather than falling into judgment and black and white thinking and extreme thinking assumptions, labels, and taking care of your nervous system after the call. Because we are co-regulating with our client, we're in a relationship with them. So that's just going to happen, and you may need to metabolize and digest if it was challenging in any way during your session.

We're going to discuss the difference between coaching and therapy, and I'll clarify for you what a Jai coach is. So I'll take this slide by slide and focus on any questions that come up for you, and make sure you bring it to your week 13 class. I want this not to be something you're spending so much time thinking and worrying over, but rather walking through the next 12 weeks with clarity and confidence that you have a particular role. You're being trained in a particular framework, and the comparisons and the insecurities can take a backseat at least temporarily while you focus on learning this method.

So coaching, and this is probably very simplified, but most people would recognize coaching as a focus that is practical and forward-building, clarifying a goal, hiring someone who has a specialty in coaching people through obstacles, whether internal or external, to arrive at that goal. You hire a health coach when you're struggling with your relationship with food. Not only are you not sure how to meal plan, what the heck to cook and eat based on your own constitution or your own physiology, but you also have a mindset around food. You have a coping mechanism with food. So a coach, if they're a good coach, is not just going to give you a seven-day meal plan and say, "Go do it," and next week we'll set a new goal. They're also going to address what is blocking you. So while they are goal-oriented and they're there for accountability—like, "Send me a picture of the meal that you cook this evening, and let's talk about and let's workshop how to cook nutritious foods during a busy week"—it's also getting them into the deeper limitations. I have that here that they don't necessarily address deeper wounds or trauma.

So perhaps the reason why this person is struggling with food is because they come from a household where they never knew when they were going to get their next meal, or they used food as a replacement



for an emotional connection because they didn't have anyone capable of seeing them and knowing them, and feeling them throughout their upbringing. A coach may or may not address that. Of course, the coaching industry is expanding rapidly, and it's becoming more and more trauma-informed, nervous system-informed, relational impact-informed. So this is stereotypical, but it would look potentially more at the mindset and less about the soil in which the mindset developed.

Therapy, by definition, is a form of treatment that is aimed at relieving emotional distress and mental health problems. So a focus on healing and integration of past experiences, trauma, unresolved emotional wounds, that can include attachment disorders, mental health diagnoses, and mental health disorders. The approaches are clinical methods: Cognitive Behavioral Therapy, trying to remember what DBT stands for, you know, if you don't Google it, EMDR, Psychodynamic Therapy, Attachment Repair, on and on and on. There are many different modalities. I am not a therapist, I am a coach and, social worker. This may include the diagnosis or treatment of mental health conditions. So they have the licensure to be able to diagnose a client and then either refer them to another method, maybe medicine to be as part of their treatment plan, or they would follow a particular treatment plan protocol depending on the framework that they've been trained under. The scope of a therapist's practice regarding parenting. People uncover why they feel or react the way they do, which builds insights, resilience, and emotional repair. So, making sense of what happened to me autobiographically has created a series of coping mechanisms, belief systems, physiological and emotional patterns, and relational patterns that I experience today.

So therapy, if it's working specifically with a parent, that therapist may or may not, I have no idea, be trained in relational centric parenting strategies and methods. It could be that this person, who is your therapist, is trained in applied behavioral science, but does not have an understanding of the nervous system on the child. That's a random example, and you may be thinking, "My therapist does," so I don't know who your therapist is. And there's, again, this is stereotypical as a way for us to create a clear delineation between coaching and therapy in order for us to make sense of, if we merge these two extremes, what's in the middle? And that's what we're looking at when we're looking at being a Jai coach, for lack of a better term. If you think of one, let me know. We can look at this like therapeutic coaching and education.

If you're being trained through Jive, you're becoming essentially a parenting educator. You're using a lot of education and teaching, you are teaching and you are coaching, and you are facilitating a therapeutic conversation. So if we were to put all of that together as a therapeutic coaching and education. So the definition of therapeutic is anything that can uplift, soothe, or provide insight. So, you are not a therapist; you may not diagnose and create a mental health treatment plan for any of your clients. You are connecting, you are listening, you are empathizing, you are asking questions, you're helping them feel more seen and known and understood. And that is therapeutic. We could call that that is relational. And, you know, painting is therapeutic, coloring in a coloring book is therapeutic. So, having a conversation with someone is therapeutic, and there is a focus as a Jai coach on being a bridge between coaching and therapy.

So there is an inner world and a past focus, and there is a present-day focus going into the future. Daniel Siegel teaches this to us in his book, *Parenting From the Inside Out*, which is about the neurobiological impact on a client's or a parent's narrative of their coherent narrative of their life. Have they made sense of their upbringing and how it's impacted them, how it's shaped them—positives, negatives, what they want to carry forward, what they want to let go of, why they have struggles with certain things, why some things come easily for them—because it gives you consciousness, gives you self-awareness and conscientiousness that you can be more present and true in the moment today. So the power of creating a coherent narrative, where that would be different with a therapy client, is that you would spend as

much time in the past autobiographical building as you needed, whereas with a coach, especially in this 12-week program, they're signing up for a limited parameter.

So we're not spending every single session trying to figure out the personality disorder of their stepfather. That's not your terrain. That is a therapist's terrain. In sessions, we can draw on a therapeutic approach, again, which is that creating a coherent narrative, why you're experiencing what you're experiencing today based on your past, how you've been imprinted upon, how you have been conditioned and trained through your relationships, through your culture, through your religion, through your demographic, the place in the world, how it has shaped you. Coaching is helping the client create specific, attainable goals. Those goals may be deeply personal and internal. Like, "I want to develop an ability to monitor my impulses and contain my impulses so I can stop patterns of emotional reactivity". Okay, that's a deeply personal goal, and in order to create that behavioral change, it's going to come from the inside out.

A component of that is going to be self-understanding. A component of that is going to be self-connection: being aware of the body, being aware of the emotions, being aware of their needs, being aware of their thoughts, being aware of their beliefs, being aware of their memories that are coming through, contributing to those impulses, and replacement skills—what to do instead, what to work on, what to practice, what to integrate over time to grow through that challenge into a new cycle.

Parents are also receiving, again, our education and our teaching. Some people might not have any idea of how the brain and body develop through attachment throughout the ages. What's the appropriate expectation? How to speak to a young child? How to focus on telling a child what to do versus telling that child to stop over and over again? Just helping to reality check parents' mindset and understanding, and perception of life and their children through developmental neuroscience.

So again, the scope is, "Why am I the way I am? Why do I do what I do?" Having compassion and empathy and understanding, having a hunger and a will to change, and creating those goals to begin taking steps towards the transformation of mindsets, behaviors, and emotional relational patterns. So the unique value of a Jai Coach is not only are we work on generational pattern transformation, but they're also leaving sessions with practical, applicable words and actions they can implement right away. And it is both intrapersonal and relational. So those actions can be the way they tend and care for themselves internally, or it can be in the parent-child dynamic: how they're speaking, preparing, preventing, intervening, and navigating the day-to-day experiences with their kids, siblings, on and on and on.

We know that there are many obstacles. So, Jai coaching, why I do it, how I can do it differently, and the education to sustain. Jai is parent-centric coaching. Because I'll talk through these, feel free to read them with me. True behavioral change that's not just replacing the behavior, but the urge and the impulse and the pattern and the unprocessed emotions are still lurking beneath the surface. But inner-outer behavioral change, pattern change, happens when a mind-body is understood, when there's a mind-body understanding of how the parent is showing up and experiencing that moment, either contributing and co-escalating the stress or being able to anchor and be the calm in the storm. A mind-body understanding of why the parent struggles to be the source of de-escalation and co-regulation versus the source of escalation.

It is parent-centric because the practice of these skills and the clarity of what it looks like and sounds like is the parent's responsibility to become aware of how they can better respond in challenging situations, so they're not feeding the—what does it look like, what does it sound like? What do they need to literally do in those moments? They need to learn, they need to become more aware and informed all around.



There is a development of mindset and emotional resilience. There is deep work like learning how to have confidence and to trust themselves, to have humility, to be willing to get it wrong, to be willing to fail, and then keep practicing. To be willing to take science and data and child development and think critically and really look inward and outward and see if what it is you're learning here aligns with what it is that you've been taught from other areas of life you deeply value. Sometimes things that are being taught through the lens of child development at the surface seem to contradict what they've been taught in other areas of their life—let's say their religion, potentially, or their family, or their culture.

So they've got to really sit with sometimes the dissonance between what science is saying children need to develop neurobiologically, neurochemically—like the infrastructure of their brain and body—and the relationships with others, not just the behaviors that they're displaying, although both matter. That sometimes goes against other areas of their life. So we're helping them sift through and really sit and be honest about how they can integrate what they're learning here into their day-to-day. We're helping them create goals, modify routines, monitor progress, monitor growth, and let go of old habits. So it's really how the client is, how the parent is, how they relate with their child? How do they speak to their child? How do they respond to their child? How do they guide, how do they teach? What does their modeling look like? And we're working on the model because the child is emulating the model, and that's our responsibility.

It may be that a child needs other support. It may be that a child needs their own therapist, their own unmet needs that need to be cared for. So we're helping the parent and the parent only because they're sitting there right in front of us. Our method centers on the mental and emotional well-being of that parent. It centers on the relational health and the emotional connection between the parent and their child, children, or family system. It centers the personal development of the adult within the context of their history and their current reality in the community. It centers on the necessary and effective leadership skills and tools that the adult needs to be competent in, or at least practice, for their child to imitate and emulate. And over time, with lots and lots and lots of practice, years of practice, become positive discipline strategies for effective guidance that are preventative, proactive, rooted in a clear, defined, well-maintained structure, and are not only relying on intervention methods. However, when an adult needs to intervene, is it helpful? Is it positive? Does it have the right amount of pressure? Is it helping a child? Is it meeting a child where they're at and giving them the real support they need to make a change?

Let's talk about the different modalities that our approach is influenced by. So we're calling on Behavioral Science. This is from the 20th century, working by understanding that all observable behavior is shaped either through positive or negative reinforcement. So it's this attempt to construct a method of engaging children's behaviors or not responding to children's behaviors that will either prevent the behavior from happening again or will encourage a behavior to happen again. What do you want to see more of? How do you respond? What you want to see less of, how do you respond?

Polyvagal Theory, I would say, is the behavioral science of the 21st century. Because it's not only looking at, "There are four reasons for every single behavior ever on planet Earth, and here are the tools that you can use to manipulate and change those behaviors to get the human being that you want." That's a lot more complex and complicated than that. There is more at play behind the child's observable behaviors that we want to be mindful and aware of: their psychological, physiological, and emotional state, the way that the brain works with emotional memory, and how it is processing emotional memory on a day-to-day basis, and day-to-day choices. So we're helping parents look beyond just four functions of behaviors. And to—there's, there's many more reasons for any moment, and we can expand our curiosity.



Authoritative Parenting is the ability to have respectful and clear authority with children, respectful and clear leadership that has the right amount of encouragement and warmth, and nurturance and priority on just being and empathizing with when that's what's needed. And attunement to that child needs more of a logical approach. They need a parent to be able to hold a standard and expectation of parameters and limits, and support that child in the ways that they need.

Interpersonal Neurobiology is the study of the brain, essentially in relationships. So the way that relationships impact your brain and your body's communication and experience. Developmental Neuroscience is how the brain develops over time, especially through critical periods, including the biological, structural, and functional changes in the brain. Mindsight and Cognitive Neuropsychology are how specific brain structures and cognitive processes work. This helps parents to understand how a child thinks, processes information, why children notice or ignore certain stimuli, and what's going on in their bias.

Attachment Theory is the bond science of the bond between the child and caregiver, especially in those critical periods, the first 1,000 days. The caregiver relating right brain to right brain in the child, the quality of emotional connection, emotional availability, and emotional warmth that has created the child's baseline understanding of trust and safety with that caregiver. Emotional Intelligence and Emotion Coaching is the ability to recognize, understand, manage, hold your own stress cycle around emotions, and to be able to effectively respond to others rather than collapsing, fighting, fleeing, fawning, or going into those nervous system stress states because being with the emotion is too overwhelming.

Positive Communication is a way of expressing needs and feelings clearly, respectfully, and empathetically. Helping parents to connect with their children while giving instructions, transitioning, resolving conflicts, intervening when the child is struggling or behaving in a way that is harmful or dangerous or loud, annoying, whatever, when you need to intervene and give that child a redirection, a reminder.

So lots of very interesting theories that we're standing on and being informed by. And you can deep dive into any of these. You can always take one or two or three of these modalities and let this be your niche. Let this be your specialty, or you can become a jack of all trades. And that's what this 12-week program is. It's a small taste and a door opening to each of these different ways of being held in our parent-child relationship by researchers and theorists surrounding us who have come before us.

So I'll discuss now permission and encouragement for you to meet your client where they are at. You're going to have lots and lots and lots of different clients, and they're going to be at different stages of growth, every one of them.

So at stage one, they don't have any awareness. They need a lot of education, a lot of handholding. They need you to break down complex information into simple guidance that they can apply right away. They need small wins. They just need to kind of get their feet underneath them here.

Stage two is they're aware of what they don't want to be doing or how they don't want to be responding, or what they could have done or should have done after the fact. You're learning and you're teaching and you're guiding, but they don't remember it in the moment. They remember, "Oh yeah, Rebecca told me that instead of this, I could have done this," but they didn't remember it. So that is, they're warming up, but it's not quite accessible.

The third stage is that they're in the active eye of learning, and they are aware of what they're doing while they're doing it. Sometimes they can choose to catch themselves in that moment. Like they started

to go toward a threat, and they said, "You know what? Hold on, let me redo that". And then they try again. Or they're aware of what they don't want to be doing while they're doing it, but they're not able to stop it quite yet. That's a really hard one.

Mastery, quote unquote, is stage four. And that is when it becomes instinctual. They're no longer fighting with themselves. They're no longer having this. "Yeah, but if we don't have the lecture right now, how are they going to learn? Well, they're going to learn when their brain works again. Can't we wait? No, we need to do it right now." And you all know this, I'm sure we have these moments. It's like, "Eh, push and pull" inside, and it is uncomfortable. But being in stage four is that that's their baseline response. A child is struggling; their baseline response is curiosity, not a threat.

So, depending on the client, they may be in stage one in this area, stage four, and on and on and on. They may be just in stage one in general, et cetera, et cetera. So you'll get to know your client. That's why we don't really start coaching until weeks four or five, six, because we're just getting to know who they are and where they're coming from and what kind of coaching they need from you based on where they are.

So jumping back to the coach and therapist conversation, we want to be mindful and aware of staying in our lane, staying in our practice, and being aware of when we're venturing into territory that is not within your training here at Jai. And I will say that this is very subjective because many, many students have had other coaching, facilitation, therapeutic, and professional training. So it's going to really be up to you because we don't have a universal code of ethics for parent coaches. These are some really great questions to keep a self-assessed eye on: "Am I staying in integrity and ethics with my clients?"

So, a few options here are to stay where you feel safe and grounded. Safe, meaning you're present. You're present too. You're not going into a fight, flight, freeze, or collapsed state. You know you're going to be activated. It's not that you're never going to have a sympathetic activation in your nervous system on calls, but you're going to know when, "This is feeling way too complex and complicated and overwhelming for me. I'm really, I'm not able to stay in myself when I'm talking to you about this. It's just too, too big for me". And you can be honest with yourself and your client about that. Ideally, we're just as transparent as we possibly can be in session. And this gets a little bit gray because you might be having a heart-pumping, brain-freezing moment even in your scope of practice just because you're scared of the pressure and the responsibility of your role. So you'll need to meet yourself where you are and be as honest as you can.

If it comes to a place where you're noticing repetitively, I would say not just one session; it was a hard session for you, you weren't expecting that, you weren't prepared for that. If it becomes recurrent, if your client is obviously in a relationship that involves psychological, verbal, emotional, or physical abuse, and that caught you off guard, and it's coming up several times, I would recommend getting out. I would recommend a couples counselor. I would contact or invite them to contact a domestic violence shelter, et cetera.

If you went too far with a client, and again, that's very subjective. What does that mean to go too far? I don't want you to be too paranoid about, "How do I know when I'm going too far?" It is, look at it like this: Am I asking questions without paying attention to the way my questions are impacting myself or my client? Meaning, is this becoming way too intense for both of us to be with? And we're just kind of spiraling out. So that's why we'll practice together in week 17, how to monitor where your client is at to the best of your ability by checking in, verbally, noticing nonverbal cues of when it becomes too much. It's, it's just too overwhelming emotionally. And we, we just need to reel it back in a little bit.

We can empower a client by spending the last few minutes of a call on, "How can you take care of yourself today? Is there anyone you can lean on this week? Is there a way that you can take care of yourself in your week?" So we don't want to be afraid of emotions. We have to practice being uncomfortable with emotions and giving ourselves the evidence that we can make it through. We can't be in avoidance of emotional discomfort, or we won't grow our capacity. So we also don't want to be afraid of our client's emotions. Like, "Oh no, they left and they were really emotional." Like, okay, yeah, they're human, they're going to be emotional. So let's talk about how they can care for themselves right after the call and how you can care for yourself after the call as well.

So I'll talk with you now about session one. And this will depend on who it is that you're working with, if you know your client, if they're a stranger. But the essence of session one is to set the container and to get to know your client, for them to get to know you, and for you to begin to establish rapport and trust as a coach-client relationship. So take care of yourself before you hop on the call, and we're listening. We're really data collecting in session one. You're going to be discussing with them what to expect. You can follow that session one outline, helping them make sense of how to succeed in this space, in this container. Who you are, who you are not. What this is, what this is not. Any questions that they have. If you have extra time in session one, I'd like to go into some storytelling. Like, "Tell me about when you met your spouse," or "Tell me about your pregnancy or the first few years," or "Tell me about your kids". I want to get to know them. I want to get to know your family. So, I like to spend it as if, "Who are you?" We're going to be sharing a lot with me in the next 12 sessions. So let's connect. I might share more about myself, my family, my story, and I can set them up logistically for what to expect going into the homework, the online portal, when we're going to meet next, expectations around boundaries, and connecting with me between calls. Is that something you allow? Are they, are they allowed to email you, WhatsApp? What, what are your rules? Everyone gets to define that for themselves.

I would say have fun. Slow down and just get to know who your client is and what they're bringing to the table. Be aware of any assumptions that you have, any judgments that you have, any fear stories that you have after leaving session one, because clients can grow or shrink depending on what's waiting for them throughout this process, from one session to the next. So don't come to any conclusions, but be curious. Be curious and open.

So you're going into week 14, which will be Setting Intentions as a Transformational Coach and a discussion of the Five D Coaching Process. I'll see you all there next week. For now, enjoy what you have moved through. Enjoy working through session one with your clients. Any questions, please bring those to your cohort call week 13, and I'll see you soon. Take care everyone

